

You know, much of the research which is our area of interest that was being done by the Agency and the whole involved sequence of activities done by the Agency, I am convinced could have been done in a legitimate way through the research programs of the National Institutes of Mental Health, other sponsored activities. I mean, that is some other question, but I think you went to an awful lot of trouble, where these things could have been.

Let me ask you specifically, on the followup of MKULTRA, are there now—I think you have answered, but I want to get a complete answer about any experimentations that are being done on human beings, whether it is drugs or behavioral alterations or patterns or any support, either directly or indirectly, being provided by the Agency in terms of any experimentation on human beings.

Admiral TURNER. There is no experimentation with drugs on human beings, witting or unwitting, being conducted in any way.

Senator KENNEDY. All right. Or being supported indirectly? I mean, are you contracted out?

Admiral TURNER. Or being in any way supported.

Senator KENNEDY. All right. How about the nondrug experimentation our Committee has seen—psychosurgery, for example, or psychological research?

Admiral TURNER. We are continually involved in what we call assessment of behavior. For instance, we are trying to continually improve our polygraph procedures to, you know, assess whether a person is lying or not. This does not involve any tampering with the individual body. This involves studying records of people's behavior under different circumstances, and so on, but it is not an experimental thing. Have I described that accurately, Al?

Mr. BRODY. Yes.

Senator KENNEDY. Well, it is limited to those areas?

Admiral TURNER. Yes; it does not involve attempting to modify behavior. It only involves studying behavior conditions, but not trying to actively modify it, as was one of the objectives of MKULTRA.

Senator KENNEDY. Well, we are scarce on time, but I am interested in the other areas besides polygraph where you are doing it. Maybe you can either respond now or submit it for the record, if you would do that. Would you provide that for the record?

Admiral TURNER. Yes.

[The material on psychological assessments follows:]

Psychological assessments are performed as a service to officers in the operations directorate who recruit and/or handle agents. Except for people involved in training courses, the subjects of the assessments are foreign nationals. The assessments are generally done to determine the most successful tactic to persuade the subject to accept covert employment by the CIA, and to make an appraisal of his reliability and truthfulness.

A majority of the work is done by a staff of trained psychologists, some of whom are stationed overseas. The assessments they do may be either direct or indirect. Direct assessments involve a personal interview of the subject by the psychologist. When possible the subject is asked to complete a formal "intelligence test" which is actually a disguised psychological test. Individuals being assessed are not given drugs, nor are they subjected to physical harassment or torture. When operating conditions are such that a face-to-face interview is not possible, the psychologist may do an indirect assessment, using as source materials descriptions of the subject by others, interviews with people who know him, specimens of his writings, etc.

The other psychological assessments involve handwriting analysis or graphological assessment. The work is done by a pair of trained graphologists, assisted by a small number of measurement technicians. They generally require at least a page of handwritten script by the subject. Measurements are made of about 30 different writing characteristics, and these are charted and furnished to the graphologist for assessments.

The psychologists also give courses in psychological assessment to group of operations officers, to sharpen their own capabilities to size up people. As part of the training course, the instructor does a psychological assessment of each student. The students are witting participants, and results are discussed with them.

It is important to reiterate that psychological assessments are only a service to the operations officers. In the final analysis, it is the responsibility of the operations officer to decide how a potential agent should be approached, or to make a judgement as to whether any agent is telling the truth.

Admiral TURNER. The kind of thing we are interested in is, what will motivate a man to become an agent of the United States in a difficult situation. We have to be familiar with that kind of attitudinal response that we can expect from people we approach to for one reason or another become our spies, but I will be happy to submit a very specific listing of these.

Senator KENNEDY. Would you do that for the committee?

In the followups, in the MKSEARCH, in the OFTEN, and the CHICKWIT, could you give us also a report on those particular programs?

Admiral TURNER. Yes, sir.

Senator KENNEDY. Did they involve experimentation, human experimentation?

Admiral TURNER. No, sir.

Senator KENNEDY. None of them?

Admiral TURNER. Let me say this, that CHICKWIT program is the code name for the CIA participation in what was basically a Department of Defense program. This program was summarized and reported to the Church committee, to the Congress, and I have since they have been rementioned in the press in the last 2 days here, I have not had time to go through and personally review them. I have ascertained that all of the files that we had and made available before are intact, and I have put a special order out that nobody will enter those files or in any way touch them without my permission at this point, but they are in the Retired Records Center outside of Washington, and they are available.

I am not prepared to give you full details on it, because I simply haven't read into that part of our history, but in addition I would suggest when we want to get into that we should get the Department of Defense in with us.

Senator KENNEDY. Well, you will supply that information to the Intelligence Committee, the relevant, I mean, the health aspects, obviously, and the research we are interested in?

Admiral TURNER. Yes, sir.

Senator KENNEDY. Will you let us know, Admiral Turner?

Admiral TURNER. I will be happy to.

[See p. 169 for the material referred to.]

Senator KENNEDY. Thank you. I am running out of time. Do you support the extension of the protection of human subjects legislation to include the CIA and the DOD? You commented favorably on that

before, and I am hopeful we can get that on the calendar early in September, and that is our strong interest.

Admiral TURNER. The CIA certainly has no objection to that proposed legislation, sir. It is not my role in the administration to be the supporter of it or the endorser of it.

Senator KENNEDY. As a personal matter, since you have reviewed these subjects, would you comment? I know it is maybe unusual, but you can understand what we are attempting to do.

Admiral TURNER. Yes, sir.

Senator KENNEDY. From your own experience in the agency, you can understand the value of it.

Just finally, in your own testimony now with this additional information, it seems quite apparent to me that you can reconstruct in very careful detail this whole project in terms of the responsible CIA officials for the program. You have so indicated in your testimony. Now with the additional information, and the people, that have been revealed in the examination of the documents, it seems to be pretty clear that you can track that whole program in very careful detail, and I would hope, you know, that you would want to get to the bottom of it, as the Congress does as well. I will come back to that in my next round. Thank you very much.

Senator INOUE. Senator Goldwater?

Senator GOLDWATER. I have no questions.

Senator INOUE. Senator Schweiker?

Senator SCHWEIKER. Thank you, Mr. Chairman.

Admiral Turner, I would like to go back to your testimony on page 12, where you discuss the contribution to the building fund of a private medical institution. You state, "Indeed, it was mentioned in a 1957 Inspector General report on the Technical Service Division of CIA, pertinent portions of which had been reviewed by the Church committee staff." I would like to have you consider this question very carefully. I served as a member of the original Church committee. My staffer did a lot of the work that you are referring to here. He made notes on the IG's report. My question to you is, are you saying that the section that specifically delineates an improper contribution was in fact given to the Church committee staff to see?

Admiral TURNER. The answer to your question is "Yes." The information that a contribution had been made was made available, to the best of my knowledge.

Senator SCHWEIKER. Only certain sections of the report were made available. The report had to be reviewed out at Langley; it was not reviewed here, and copies were not given to us here. I just want you to carefully consider what you are saying, because the only record we have are the notes that the staff took on anything that was of significance.

Admiral TURNER. My understanding was that Mr. Maxwell was shown the relevant portion of this report that disclosed that the contribution had been made.

Senator SCHWEIKER. To follow this up further, I'd like to say that I think there was a serious flaw in the way that the IG report was handled and the Church committee was limited. I am not making any accusations, but because of limited access to the report, we have a situ-

ation where it is not even clear whether we actually saw that material or not, simply because we could not keep a copy of the report under the procedures we had to follow. We were limited by notetaking, and so it is rather ambiguous as to just what was seen and what was not seen. I certainly hope that the new Intelligence Committee will not be bound by procedures that so restrict its ability to exercise effective oversight.

I have a second question. Does it concern you, Admiral, that we used a subterfuge which resulted in the use of Federal construction grant funds to finance facilities for these sorts of experiments on our own people? Because as I understand what you are saying, while the CIA maybe only put up \$375,000, this triggered a response on the part of the Federal Government to provide on a good faith basis matching hospital funds at the same level. We put up more than \$1 million of matching funds, some based on an allegedly private donation which was really CIA money.

Isn't there something basically wrong with that?

Admiral TURNER. I certainly believe there is. As I stated, the General Counsel of the CIA at that time rendered a legal opinion that this was a legal undertaking, and again I am hesitant to go back and revisit the atmosphere, the laws, the attitudes at that time, so whether the counsel was on good legal ground or not, I am not enough of a lawyer to be sure, but it certainly would occur to me if it happened today as a very questionable activity.

Senator SCHWEIKER. Well, I think those of us who worked on and amended the Hill-Burton Act and other hospital construction assistance laws over the years, would have a rather different opinion on the legal intent or object of Congress in passing laws to provide hospital construction project money. These funds weren't intended for this.

It reminds me a little bit of the shellfish toxin situation which turned up when I was on the Church committee. The Public Health Service was used to produce a deadly poison with Public Health money. Here we are using general hospital construction money to carry on a series of drug experiments.

Admiral TURNER. Excuse me, sir. If I could just be, I think, accurate, I don't think any of this \$375,000 or the matching funds were used to conduct drug experiments. They were used to build the hospital. Now, the CIA then put more money into a foundation that was conducting research on the CIA's behalf supposedly in that hospital, so the intent was certainly there, but the money was not used for experimentation.

Senator SCHWEIKER. Well, I understand it was used for bricks and mortar, but the bricks were used to build the facility where the experiments were carried on; were they not?

Admiral TURNER. We do not have positive evidence that they were. It certainly would seem that that was the intent, but I do not want to draw inferences here—

Senator SCHWEIKER. Well, why else would they give this money for the building fund if the building was not used for a purpose that benefited the CIA program?

Admiral TURNER. I certainly draw the inference that the CIA expected to benefit from it, and some of the wording says the General

Counsel's opinion was that this was legal only if the CIA was going to derive adequate benefit from it, but, sir, there is no evidence of what benefit was derived.

Senator SCHWEIKER. There must have been some pretty good benefits at stake. The Atomic Energy Commission was to bear a share of the cost, and when they backed out for some reason or another, the CIA picked up part of their tab. So, at two different points there were indications that CIA decisionmakers thought there was great benefit to be derived from whatever happened within the brick and mortar walls of that facility.

Admiral TURNER. You are absolutely right. I am only taking the position that I cannot substantiate that there was benefit derived.

Senator SCHWEIKER. The agreement documents say that the CIA would have access to one-sixth of the space involved in the construction of the wing, so how would you enter into an agreement that specifically says that you will have access to and use of one-sixth of the space and not perform something in that space? I cannot believe it was empty.

Admiral TURNER. Sir, I am not disputing you at all, but both of us are saying that the inference is that one-sixth of the space was used, that experimentation was done, and so on, but there is no factual evidence of what went on as a result of that payment or what went on in that hospital. It is just missing. It is not that it didn't happen.

Senator SCHWEIKER. Admiral Turner, one other—

Senator KENNEDY. Would the Senator yield on that point?

Senator SCHWEIKER. I understand that in the agency's documents on the agreement it was explicitly stated that one-sixth of the facility would be designated for CIA use and made available for CIA research. Are you familiar—

Mr. BRODY. Senator, as I recall, you are right in that there is a mention of one-sixth, but any mention at all has to do with planning. There are no subsequent reports as to what happened after the construction took place.

Senator SCHWEIKER. Admiral Turner, I read in the New York Times that part of this series of MKULTRA experiments involved an arrangement with the Federal Bureau of Narcotics to test LSD surreptitiously on unwitting patrons in bars in New York and San Francisco. Some of the subjects became violently ill and were hospitalized. I wonder if you would just briefly describe what we were doing there and how it was carried out? I assume it was through a safe house operation. I don't believe your statement went into much detail.

Admiral TURNER. I did mention the safe house operation in my statement, sir, and that is how these were carried out. What we have learned from the new documentation is the location and the dates at which the safe houses were run by the CIA and the identification of three individuals who were associated with running those safe houses. We know something about the construction work that was done in them because there were contracts for this. Beyond that, we are pretty much drawing inferences as to the things that went on as to what you are saying here.

Senator SCHWEIKER. Well, the subjects were unwitting. You can infer that much, right?

Admiral TURNER. Right.

Senator SCHWEIKER. If you happened to be at the wrong bar at the wrong place and time, you got it.

Mr. BRODY. Senator, that would be—contacts were made, as we understand it, in bars, et cetera, and then the people may have been invited to these safe houses. There really isn't any indication as to the fact that this took place in the bars.

Admiral TURNER. We are trying to be very precise with you, sir, and not draw an inference here. There are 6 cases of these 149 where we have enough evidence in this new documentation to substantiate that there was unwitting testing and some of that involves these safe houses. There are other cases where it is ambiguous as to whether the testing was witting or voluntary. There are others where it was clearly voluntary.

Senator SCHWEIKER. Of course, after a few drinks, it is questionable whether informed consent means anything to a person in a bar anyway.

Admiral TURNER. Well, we don't have any indication that all these cases where it is ambiguous involved drinking of any kind. There are cases in penal institutions where it is not clear whether the prisoner was given a choice or not. I don't know that he wasn't given a choice, but I don't positively know that he was, and I classify that as an ambiguous incident.

Senator INOUE. Your time is up, Senator.

Senator Huddleston?

Senator HUDDLESTON. Thank you, Mr. Chairman.

Admiral Turner, you stated in your testimony that you are convinced there was no attempt to conceal this recently discovered documentation during the earlier searches. Did you question the individuals connected with the earlier search before you made that judgment?

Admiral TURNER. Yes; I haven't, I don't think, questioned everybody who looked in the files or is still on our payroll who looked in the files back in 1975, but Mr. Laubinger on my left is the best authority on this, and I have gone over it with him in some detail.

Senator HUDDLESTON. But you have inquired, you think, sufficiently to assure yourself that there was no intent on the part of any person to conceal these records from the previous committee?

Admiral TURNER. I am persuaded of that both by my questioning of people and by the circumstances and the way in which these documents were filed, by the fact which I did not and should have mentioned in my testimony, that these were not the official files. The ones that we have received or retrieved were copies of files that were working files that somebody had used, and therefore were slipped into a different location, and again I say to you, sir, I can't imagine their deliberately concealing these particular files and revealing the other things that they did reveal in 1975. I don't see the motive for that, because these are not that damning compared with the overall material that was provided.

Senator HUDDLESTON. Is this the kind of operation that if it were continuing now or if there were anything similar to it, that you would feel compelled to report to the Select Committee on Intelligence?

Admiral TURNER. Yes, sir. You mean, if I discovered that something like this were going on without my knowledge? Yes, I would feel absolutely the requirement to—

Senator HUDDLESTON. But if it were going on with your knowledge, would you report it to the committee? I assume you would.

Admiral TURNER. Yes. Well, it would not be going on with my knowledge, but theoretically the answer is yes, sir.

Senator HUDDLESTON. Well, then, what suggestions would you have as we devise charters for the various intelligence agencies? What provision would you suggest to prohibit this kind of activity from taking place? Would you suggest that it ought to be specifically outlined in a statutory charter setting out the parameters of the permissible operation of the various agencies?

Admiral TURNER. I think that certainly is something we must consider as we look at the legislation for charters. I am not on the face of it opposed to it. I think we would have to look at the particular wording as we are going to have to deal with the whole charter issue as to exactly how precise you want to be in delineating restraints and curbs on the intelligence activities.

Senator HUDDLESTON. In the case of sensitive type operations, which this certainly was, which might be going on today, is the oversight activity of the agency more intensive now than it was at that time?

Admiral TURNER. Much more so. I mean, I have briefed you, sir, and the committee on our sensitive operations. We have the Intelligence Oversight Board. We have a procedure in the National Security Council for approval of very sensitive operations. I think the amount of spotlight focused on these activities is many, manyfold what it was in these 12 to 24 years ago.

Senator HUDDLESTON. How about the record keeping?

Admiral TURNER. Yes; I can't imagine anyone having the gall to think that he can just blithely destroy records today with all of the attention that has come to this, and certainly we are emphasizing that that is not the case.

Senator HUDDLESTON. Admiral, I was particularly interested in the activity that took place at the U.S. Public Health Service Hospital at Lexington, Ky., in which a Dr. Harris Isbell conducted experiments on people who were presumably patients there. There was a narcotics institution, I take it, and Dr. Isbell was, according to the New York Times story, carrying on a secret series of correspondence with an individual at the agency by the name of Ray. Have you identified who that person is?

Admiral TURNER. Sir, I find myself in a difficult position here at a public hearing to confirm or deny these names in view of my legal responsibilities under the Privacy Act not to disclose the names of individuals here.

Senator HUDDLESTON. I am just asking you if you have identified the person referred to in that article as Ray. I am not asking you who he was. I just want to know if you know who he is.

Admiral TURNER. No. I am sorry, was this W-r-a-y or R-a-y?

Senator HUDDLESTON. It is listed in the news article as R-a-y, in quotations.

Admiral TURNER. No, sir, we have not identified him.

Senator HUDDLESTON. So you have no knowledge of whether or not he is still a member of your staff or connected with the Agency in any way. Have you attempted to identify him?

[Pause.]

Admiral TURNER. Senator, we have a former employee whose first name is Ray who may have had some connection with these activities.

Senator HUDDLESTON. You suspect that but you have not verified that at this time, or at least you are not in a position to indicate that you have verified it?

Admiral TURNER. That is correct.

Senator HUDDLESTON. Thank you.

Thank you, Mr. Chairman.

Senator INOUE. Senator Wallop?

Senator WALLOP. Thank you, Mr. Chairman.

Admiral TURNER, not all of the—and in no way trying to excuse you of the hideous nature of some of these projects, but not all of the projects under MKULTRA are of a sinister or even a moral nature. Is that a fair statement?

Admiral TURNER. That is correct.

Senator WALLOP. Looking down through some of these 17 projects not involving human testing, aspects of the magician's art, it doesn't seem as though there is anything very sinister about that. Studies of human behavior and sleep research, library searches. Now, those things in their way are still of interest, are they not, to the process of intelligence gathering?

Admiral TURNER. Yes, sir. I have not tried to indicate that we either are not doing or would not do any of the things that were involved in MKULTRA, but when it comes to the witting or unwitting testing of people with drugs, that is certainly verboten, but there are other things.

Senator WALLOP. Even with volunteer patients? I mean, I am not trying to put you on the spot to say whether it is going on, but I mean, it is not an uncommon thing, is it, in the prisons of the United States for the Public Health Service to conduct various kinds of experiments with vaccines and, say, sunburn creams? I know in Arizona they have done so.

Admiral TURNER. My understanding is, lots of that is authorized, but I am not of the opinion that this is not the CIA's business, and that if we need some information in that category, I would prefer to go to the other appropriate authorities of the Government and ask them to get it for us rather than to in any way—

Senator WALLOP. Well, you know, you have library searches and attendants at the national seminars. This is why I wanted to ask you if the bulk of these projects were in any way the kinds of things that the Agency might not do now. A President would not have been horrified by the list of the legitimate types of things. Isn't that probably the case?

Admiral TURNER. Yes, sir.

Senator WALLOP. And if it did in fact appear in the IG report, is there any reason to suppose that the President did not know of this project? You said there was no reason to suppose that he did, but let me reverse that. Is there any reason to suppose that they did not?

Admiral TURNER. No.

Senator WALLOP. Well, you know, I just cannot imagine you or literally anybody undertaking projects of the magnitude of dollars here and just not knowing about it, not informing your superior that

these were going on, especially when certain items of it appear in the Inspector General's report on budget matters.

Admiral TURNER. Well, I find it difficult when it is that far back to hypothesize what the procedures that the Director was using in terms of informing his superiors were. It is quite a different climate from today, and I think we do a lot more informing today than they did back then, but I find it very difficult to guess what the level of knowledge was.

Senator WALLOP. I am really not asking you to second-guess it, but it just seems to me that, while the past is past, and thank goodness we are operating under different sets of circumstances, I think it is naive for us to suppose that these things were conducted entirely without the knowledge of the Presidents of the United States during those times. It is just the kinds of research information that was being sought was vital to the United States, not the means, but the information that they were trying to find.

Admiral TURNER. I am sorry. Your question is, was this vital? Did we view it as vital?

Senator WALLOP. Well, your implication at the beginning was that it was a response to the kinds of behavior that were seen in Cardinal Mindszenty's trial and other things. I mean, somebody must have thought that this was an important defensive reaction, if nothing else, on the part of the United States.

Admiral TURNER. Yes, sir, I am sure they did, but again I just don't know how high that permeated the executive branch.

Senator WALLOP. But the kinds of information are still important to you. I mean, I am not suggesting that anyone go back and do that kind of thing again, but I'm certain it would be of use to you to know what was going to happen to one of your agents assuming someone had put one of these things into his bloodstream, or tried to modify his behavior.

Admiral TURNER. Absolutely, and you know, we would be very concerned if we thought there were things like truth serums or other things that our agents or others could be subjected to by use or improper use of drugs by other powers against our people or agents.

Senator WALLOP. Are there? I don't ask you to name them, but are there such serums?

Admiral TURNER. I don't know of them if there are. I would have to answer that for the record, sir.

Senator WALLOP. I would appreciate that.

[The material referred to follows:]

"TRUTH" DRUGS IN INTERROGATION

The search for effective aids to interrogation is probably as old as man's need to obtain information from an uncooperative source and as persistent as his impatience to shortcut any tortuous path. In the annals of police investigation, physical coercion has at times been substituted for painstaking and time-consuming inquiry in the belief that direct methods produce quick results. Sir James Stephens, writing in 1883, rationalizes a grisly example of "third degree" practices by the police of India: "It is far pleasanter to sit comfortably in the shade rubbing red pepper in a poor devil's eyes than to go about in the sun hunting up evidence."

More recently, police officials in some countries have turned to drugs for assistance in extracting confessions from accused persons, drugs which are assumed

to relax the individual's defenses to the point that he unknowingly reveals truths he has been trying to conceal. This investigative technique, however humanitarian as an alternative to physical torture, still raises serious questions of individual rights and liberties. In this country, where drugs have gained only marginal acceptance in police work, their use has provoked cries of "psychological third degree" and has precipitated medico-legal controversies that after a quarter of a century still occasionally flare into the open.

The use of so-called "truth" drugs in police work is similar to the accepted psychiatric practice of narco-analysis; the difference in the two procedures lies in their different objectives. The police investigator is concerned with empirical truth that may be used against the suspect, and therefore almost solely with *probative* truth: the usefulness of the suspect's revelations depends ultimately on their acceptance in evidence by a court of law. The psychiatrist, on the other hand, using the same "truth" drugs in diagnosis and treatment of the mentally ill, is primarily concerned with *psychological* truth or psychological reality rather than empirical fact. A patient's aberrations are reality for him at the time they occur, and an accurate account of these fantasies and delusions, rather than reliable recollection of past events, can be the key to recovery.

The notion of drugs capable of illuminating hidden recesses of the mind, helping to heal the mentally ill and preventing or reversing the miscarriage of justice, has provided an exceedingly durable theme for the press and popular literature. While acknowledging that "truth serum" is a misnomer twice over—the drugs are not sera and they do not necessarily bring forth probative truth—journalistic accounts continue to exploit the appeal of the term. The formula is to play up a few spectacular "truth" drug successes and to imply that the drugs are more maligned than need be and more widely employed in criminal investigation than can officially be admitted.

Any technique that promises an increment of success in extracting information from an uncompliant source is *ipso facto* of interest in intelligence operations. If the ethical considerations which in Western countries inhibit the use of narco-interrogation in police work are felt also in intelligence, the Western services must at least be prepared against its possible employment by the adversary. An understanding of "truth" drugs, their characteristic actions, and their potentialities, positive and negative, for eliciting useful information is fundamental to an adequate defense against them.

This discussion, meant to help toward such an understanding, draws primarily upon openly published materials. It has the limitations of projecting from criminal investigative practices and from the permissive atmosphere of drug psychotherapy.

SCOPOLAMINE AS "TRUTH SERUM"

Early in this century physicians began to employ scopolamine, along with morphine and chloroform, to induce a state of "twilight sleep" during childbirth. A constituent of henbane, scopolamine was known to produce sedation and drowsiness, confusion and disorientation, incoordination, and amnesia for events experienced during intoxication. Yet physicians noted that women in twilight sleep answered questions accurately and often volunteered exceedingly candid remarks.

In 1922 it occurred to Robert House, a Dallas, Texas obstetrician, that a similar technique might be employed in the interrogation of suspected criminals, and he arranged to interview under scopolamine two prisoners in the Dallas county jail whose guilt seemed clearly confirmed. Under the drug, both men denied the charges on which they were held; and both, upon trial, were found not guilty. Enthusiastic at this success, House concluded that a patient under the influence of scopolamine "cannot create a lie . . . and there is no power to think or reason." [14] His experiment and this conclusion attracted wide attention, and the idea of a "truth" drug was thus launched upon the public consciousness.

The phrase "truth serum" is believed to have appeared first in a news report of House's experiment in the *Los Angeles Record*, sometime in 1922. House resisted the term for a while but eventually came to employ it regularly himself. He published some eleven articles on scopolamine in the years 1921-1929, with a noticeable increase in polemical zeal as time went on. What had begun as something of a scientific statement turned finally into a dedicated crusade by the "father of truth serum" on behalf of his offspring, wherein he was "grossly indulgent of its wayward behavior and stubbornly proud of its minor achievements." [11]

Only a handful of cases in which scopolamine was used for police interrogation came to public notice, though there is evidence suggesting that some police forces may have used it extensively. [2, 16] One police writer claims that the *threat* of scopolamine interrogation has been effective in extracting confessions from criminal suspects, who are told they will first be rendered unconscious by chloral hydrate placed covertly in their coffee or drinking water. [16]

Because of a number of undesirable side effects, scopolamine was shortly disqualified as a "truth" drug. Among the most disabling of the side effects are hallucinations, disturbed perception, somnolence, and physiological phenomena such as headache, rapid heart, and blurred vision, which distract the subject from the central purpose of the interview. Furthermore, the physical action is long, far outlasting the psychological effects. Scopolamine continues, in some cases, to make anesthesia and surgery safer by drying the mouth and throat and reducing secretions that might obstruct the air passages. But the fantastically, almost painfully, dry "desert" mouth brought on by the drug is hardly conducive to free talking, even in a tractable subject.

THE BARBITURATES

The first suggestion that drugs might facilitate communication with emotionally disturbed patients came quite by accident in 1916. Arthur S. Lovenhart and his associates at the University of Wisconsin, experimenting with respiratory stimulants, were surprised when, after an injection of sodium cyanide, a catatonic patient who had long been mute and rigid suddenly relaxed, opened his eyes, and even answered a few questions. By the early 1930's a number of psychiatrists were experimenting with drugs as an adjunct to established methods of therapy.

At about this time police officials, still attracted by the possibility that drugs might help in the interrogation of suspects and witnesses, turned to a class of depressant drugs known as the barbiturates. By 1935 Clarence W. Muehlberger, head of the Michigan Crime Detection Laboratory at East Lansing, was using barbiturates on reluctant suspects, though police work continued to be hampered by the courts' rejection of drug-induced confessions except in a few carefully circumscribed instances.

The barbiturates, first synthesized in 1903, are among the oldest of modern drugs and the most versatile of all depressants. In this half-century some 2,500 have been prepared, and about two dozen of these have won an important place in medicine. An estimated three to four billion doses of barbiturates are prescribed by physicians in the United States each year, and they have come to be known by a variety of commercial names and colorful slang expressions: "goof-balls," Luminal, Nembutal, "red devils," "yellow jackets," "pink ladies," etc. Three of them which are used in narcoanalysis and have seen service as "truth" drugs are sodium amytal (amobarbital), pentothal sodium (thiopental), and to a lesser extent secenal (secobarbital).

As one pharmacologist explains it, a subject coming under the influence of a barbiturate injected intravenously goes through all the stages of progressive drunkenness, but the time scale is on the order of minutes instead of hours. Outwardly the sedation effect is dramatic, especially if the subject is a psychiatric patient in tension. His features slacken, his body relaxes. Some people are momentarily excited; a few become silly and giggly. This usually passes, and most subjects fall asleep, emerging later in disoriented semi-wakefulness.

The descent into narcosis and beyond with progressively larger doses can be divided as follows:

- I. Sedative stage.
- II. Unconsciousness, with exaggerated reflexes (hyperactive stage).
- III. Unconsciousness, without reflex even to painful stimuli.
- IV. Death.

Whether all these stages can be distinguished in any given subject depends largely on the dose and the rapidity with which the drug is induced. In anesthesia, stages I and II may last only two or three seconds.

The first or sedative stage can be further divided:

- Plane 1. No evident effect, or slight sedative effect.
- Plane 2. Cloudiness, calmness, amnesia. (Upon recovery, the subject will not remember what happened at this or "lower" planes or stages.)
- Plane 3. Slurred speech, old thought patterns disrupted, inability to integrate or learn new patterns. Poor coordination. Subject becomes unaware of painful stimuli.

Plane 3 is the psychiatric "work" stage. It may last only a few minutes, but it can be extended by further slow injection of drug. The usual practice is to bring the subject quickly to Stage II and to conduct the interview as he passes back into the sedative stage on the way to full consciousness.

CLINICAL AND EXPERIMENTAL STUDIES

The general abhorrence in Western countries for the use of chemical agents "to make people do things against their will" has precluded serious systematic study (at least as published openly) of the potentialities of drugs for interrogation. Louis A. Gottschalk, surveying their use in information-seeking interviews,[13] cites 136 references; but only two touch upon the extraction of intelligence information, and one of these concludes merely that Russian techniques in interrogation and indoctrination are derived from age-old police methods and do not depend on the use of drugs. On the validity of confessions obtained with drugs, Gottschalk found only three published experimental studies that he deemed worth reporting.

One of these reported experiments by D. P. Morris in which intravenous sodium amytal was helpful in detecting malingerers.[12] The subjects, soldiers, were at first sullen, negativistic, and non-productive under amytal, but as the interview proceeded they revealed the fact of and causes for their malingering. Usually the interviews turned up a neurotic or psychotic basis for the deception.

The other two confession studies, being more relevant to the highly specialized, untouched area of drugs in intelligence interrogation, deserve more detailed review.

Gerson and Victoroff[12] conducted amytal interviews with 17 neuropsychiatric patients, soldiers who had charges against them, at Tilton General Hospital, Fort Dix. First they were interviewed without amytal by a psychiatrist, who, neither ignoring nor stressing their situation as prisoners or suspects under scrutiny, urged each of them to discuss his social and family background, his army career, and his version of the charges pending against him.

The patients were told only a few minutes in advance that narcoanalysis would be performed. The doctor was considerate, but positive and forthright. He indicated that they had no choice but to submit to the procedure. Their attitudes varied from unquestioning compliance to downright refusal.

Each patient was brought to complete narcosis and permitted to sleep. As he became semiconscious and could be stimulated to speak, he was held in this stage with additional amytal while the questioning proceeded. He was questioned first about innocuous matters from his background that he had discussed before receiving the drug. Whenever possible, he was manipulated into bringing up himself the charges pending against him before being questioned about them. If he did this in a too fully conscious state, it proved more effective to ask him to "talk about that later" and to interpose a topic that would diminish suspicion, delaying the interrogation on his criminal activity until he was back in the proper stage of narcosis.

The procedure differed from therapeutic narcoanalysis in several ways: the setting, the type of patients, and the kind of "truth" sought. Also, the subjects were kept in twilight consciousness longer than usual. This state proved richest in yield of admissions prejudicial to the subject. In it his speech was thick, mumbling, and disconnected, but his discretion was markedly reduced. This valuable interrogation period, lasting only five to ten minutes at a time, could be reinduced by injecting more amytal and putting the patient back to sleep.

The interrogation technique varied from case to case according to background information about the patient, the seriousness of the charges, the patient's attitude under narcosis, and his rapport with the doctor. Sometimes it was useful to pretend, as the patient grew more fully conscious, that he had already confessed during the amnesic period of the interrogation, and to urge him, while his memory and sense of self-protection were still limited, to continue to elaborate the details of what he had "already described." When it was obvious that a subject was withholding the truth, his denials were quickly passed over and ignored, and the key questions would be reworded in a new approach.

Several patients revealed fantasies, fears, and delusions approaching delirium, much of which could readily be distinguished from reality. But sometimes there was no way for the examiner to distinguish truth from fantasy except by reference to other sources. One subject claimed to have a child that did not exist,

another threatened to kill on sight a stepfather who had been dead a year, and yet another confessed to participating in a robbery when in fact he had only purchased goods from the participants. Testimony concerning dates and specific places was untrustworthy and often contradictory because of the patient's loss of time-sense. His veracity in citing names and events proved questionable. Because of his confusion about actual events and what he thought or feared had happened, the patient at times managed to conceal the truth unintentionally.

As the subject revived, he would become aware that he was being questioned about his secrets and, depending upon his personality, his fear of discovery, or the degree of his disillusionment with the doctor, grow negativistic, hostile, or physically aggressive. Occasionally patients had to be forcibly restrained during this period to prevent injury to themselves or others as the doctor continued to interrogate. Some patients, moved by fierce and diffuse anger, the assumption that they had already been tricked into confessing, and a still limited sense of discretion, defiantly acknowledged their guilt and challenged the observer to "do something about it." As the excitement passed, some fell back on their original stories and others verified the confessed material. During the follow-up interview nine of the 17 admitted the validity of their confessions; eight repudiated their confessions and reaffirmed their earlier accounts.

With respect to the reliability of the results of such interrogation, Gerson and Victoroff conclude that persistent, careful questioning can reduce ambiguities in drug interrogation, but cannot eliminate them altogether.

At least one experiment has shown that subjects are capable of maintaining a lie while under the influence of a barbiturate. Redlich and his associates at Yale[25] administered sodium amytal to nine volunteers, students and professionals, who had previously, for purposes of the experiment, revealed shameful and guilt-producing episodes of their past and then invented false self-protective stories to cover them. In nearly every case the cover story retained some elements of the guilt inherent in the true story.

Under the influence of the drug, the subjects were crossexamined on their cover stories by a second investigator. The results, though not definitive, showed that normal individuals who had good defenses and no overt pathological traits could stick to their invented stories and refuse confession. Neurotic individuals with strong unconscious self-punitive tendencies, on the other hand, both confessed more easily and were inclined to substitute fantasy for the truth, confessing to offenses never actually committed.

In recent years drug therapy has made some use of stimulants, most notably amphetamine (Benzedrine) and its relative methamphetamine (Methedrine). These drugs, used either alone or following intravenous barbiturates, produce an outpouring of ideas, emotions, and memories which has been of help in diagnosing mental disorders. The potential of stimulants in interrogation has received little attention, unless in unpublished work. In one study of their psychiatric use Brussel *et al.* [7] maintain that methedrine gives the liar no time to think or to organize his deceptions. Once the drug takes hold, they say, an insurmountable urge to pour out speech traps the malingerer. Gottschalk, on the other hand, says that this claim is extravagant, asserting without elaboration that the study lacked proper controls.[13] It is evident that the combined use of barbiturates and stimulants, perhaps along with ataraxics (tranquillizers), should be further explored.

OBSERVATIONS FROM PRACTICE

J. M. MacDonald, who as a psychiatrist for the District Courts of Denver has had extensive experience with narcoanalysis, says that drug interrogation is of doubtful value in obtaining confessions to crimes. Criminal suspects under the influence of barbiturates may deliberately withhold information, persist in giving untruthful answers, or falsely confess to crimes they did not commit. The psychopathic personality, in particular, appears to resist successfully the influence of drugs.

MacDonald tells of a criminal psychopath who, having agreed to narco-interrogation, received 1.5 grams of sodium amytal over a period of five hours. This man feigned amnesia and gave a false account of a murder. "He displayed little or no remorse as he (falsely) described the crime, including burial of the body. Indeed he was very self-possessed and he appeared almost to enjoy the examination. From time to time he would request that more amytal be injected." [21]

MacDonald concludes that a person who gives false information prior to re-

ceiving drugs is likely to give false information also under narcosis, that the drugs are of little value for revealing deceptions, and that they are more effective in releasing unconsciously repressed material than in evoking consciously suppressed information.

Another psychiatrist known for his work with criminals, L. Z. Freedman, gave sodium amytal to men accused of various civil and military antisocial acts. The subjects were mentally unstable, their conditions ranging from character disorders to neuroses and psychoses. The drug interviews proved psychiatrically beneficial to the patients, but Freedman found that his view of objective reality was seldom improved by their revelations. He was unable to say on the basis of the narco-interrogation whether a given act had or had not occurred. Like MacDonald, he found that psychopathic individuals can deny to the point of unconsciousness crimes that every objective sign indicates they have committed.[10]

F. G. Inbau, Professor of Law at Northwestern University, who has had considerable experience observing and participating in "truth" drug tests, claims that they are occasionally effective on persons who would have disclosed the truth anyway had they been properly interrogated, but that a person determined to lie will usually be able to continue the deception under drugs.

The two military psychiatrists who made the most extensive use of narcoanalysis during the war years, Roy R. Grinker and John C. Spiegel, concluded that in almost all cases they could obtain from their patients essentially the same material and give them the same emotional release by therapy without the use of drugs, provided they had sufficient time.

The essence of these comments from professionals of long experience is that drugs provide rapid access to information that is psychiatrically useful but of doubtful validity as empirical truth. The same psychological information and a less adulterated empirical truth can be obtained from fully conscious subjects through non-drug psychotherapy and skillful police interrogation.

APPLICATION TO CI INTERROGATION

The almost total absence of controlled experimental studies of "truth" drugs and the spotty and anecdotal nature of psychiatric and police evidence require that extrapolations to intelligence operations be made with care. Still, enough is known about the drugs' action to suggest certain considerations affecting the possibilities for their use in interrogations.

It should be clear from the foregoing that at best a drug can only serve as an aid to an interrogator who has a sure understanding of the psychology and techniques of normal interrogation. In some respects, indeed, the demands on his skill will be increased by the baffling mixture of truth and fantasy in drug-induced output. And the tendency against which he must guard in the interrogate to give the responses that seem to be wanted without regard for facts will be heightened by drugs: the literature abounds with warnings that a subject in narcosis is extremely suggestible.

It seems possible that this suggestibility and the lowered guard of the narcotic state might be put to advantage in the case of a subject feigning ignorance of a language or some other skill that had become automatic with him. Lipton[20] found sodium amytal helpful in determining whether a foreign subject was merely pretending not to understand English. By extension, one can guess that a drugged interrogatee might have difficulty maintaining the pretense that he did not comprehend the idiom of a profession he was trying to hide.

There is the further problem of hostility in the interrogator's relationship to a resistance source. The accumulated knowledge about "truth" drug reaction has come largely from patient-physician relationships of trust and confidence. The subject in narcoanalysis is usually motivated *a priori* to cooperate with the psychiatrist, either to obtain relief from mental suffering or to contribute to a scientific study. Even in police work, where an atmosphere of anxiety and threat may be dominant, a relationship of trust frequently asserts itself: the drug is administered by a medical man bound by a strict code of ethics; the suspect agreeing to undergo narcoanalysis in a desperate bid for corroboration of his testimony trusts both drug and psychiatrist, however apprehensively; and finally, as Freedman and MacDonald have indicated, the police psychiatrist frequently deals with a "sick" criminal, and some order of patient-physician relationship necessarily evolves.

Rarely has a drug interrogation involved "normal" individuals in a hostile or genuinely threatening milieu. It was from a non-threatening experimental setting that Eric Lindemann could say that his "normal" subjects "reported a general sense of euphoria, ease and confidence, and they exhibited a marked increase in talkativeness and communicability." [18] Gerson and Victoroff list poor doctor-patient rapport as one factor interfering with the completeness and authenticity of confessions by the Fort Dix soldiers, caught as they were in a command performance and told they had no choice but to submit to narco-interrogation.

From all indications, subject-interrogator rapport is usually crucial to obtaining the psychological release which may lead to unguarded disclosures. Role-playing on the part of the interrogator might be a possible solution to the problem of establishing rapport with a drugged subject. In therapy, the British narco-analyst William Sargent recommends that the therapist deliberately distort the facts of the patient's life-experience to achieve heightened emotional response and abreaction. [27] In the drunken state of narcoanalysis patients are prone to accept the therapist's false constructions. There is reason to expect that a drugged subject would communicate freely with an interrogator playing the role of relative, colleague, physician, immediate superior, or any other person to whom his background indicated he would be responsive.

Even when rapport is poor, however, there remains one facet of drug action eminently exploitable in interrogation—the fact that subjects emerge from narcosis feeling they have revealed a great deal, even when they have not. As Gerson and Victoroff demonstrated at Fort Dix, this psychological set provides a major opening for obtaining genuine confessions.

POSSIBLE VARIATIONS

In studies by Beecher and his associates, [3-6] one-third to one-half the individuals tested proved to be placebo reactors, subjects who respond with symptomatic relief to the administration of any syringe, pill, or capsule, regardless of what it contains. Although no studies are known to have been made of the placebo phenomenon as applied to narco-interrogation, it seems reasonable that when a subject's sense of guilt interferes with productive interrogation, a placebo for pseudo-narcosis could have the effect of absolving him of the responsibility for his acts and thus clear the way for free communication. It is notable that placebos are most likely to be effective in situations of stress. The individuals most likely to react to placebos are the more anxious, more self-centered, more dependent on outside stimulation, those who express their needs more freely socially, talkers who drain off anxiety by conversing with others. The non-reactors are those clinically more rigid and with better than average emotional control. No sex or I.Q. differences between reactors and non-reactors have been found.

Another possibility might be the combined use of drugs with hypnotic trance and post-hypnotic suggestion: hypnosis could presumably prevent any recollection of the drug experience. Whether a subject can be brought to trance against his will or unaware, however, is a matter of some disagreement. Orne, in a survey of the potential uses of hypnosis in interrogation, [23] asserts that it is doubtful, despite many apparent indications to the contrary, that trance can be induced in resistant subjects. It may be possible, he adds, to hypnotize a subject unaware, but this would require a positive relationship with the hypnotist not likely to be found in the interrogation setting.

In medical hypnosis, pentothal sodium is sometimes employed when only light trance has been induced and deeper narcosis is desired. This procedure is a possibility for interrogation, but if a satisfactory level of narcosis could be achieved through hypnotic trance there would appear to be no need for drugs.

DEFENSIVE MEASURES

There is no known way of building tolerance for a "truth" drug without creating a disabling addiction, or of arresting the action of a barbiturate once induced. The only full safeguard against narco-interrogation is to prevent the administration of the drug. Short of this, the best defense is to make use of the same knowledge that suggests drugs for offensive operations: if a subject knows that on emerging from narcosis he will have an exaggerated notion of how much he has revealed he can better resolve to deny he has said anything.

The disadvantages and shortcomings of drugs in offensive operations become positive features of the defense posture. A subject in narco-interrogation is intoxicated, wavering between deep sleep and semi-wakefulness. His speech is garbled and irrational, the amount of output drastically diminished. Drugs disrupt established thought patterns, including the will to resist, but they do so indiscriminately and thus also interfere with the patterns of substantive information the interrogator seeks. Even under the conditions most favorable for the interrogator, output will be contaminated by fantasy, distortion, and untruth.

Possibly the most effective way to arm oneself against narco-interrogation would be to undergo a "dry run." A trial drug interrogation with output taped for playback would familiarize an individual with his own reactions to "truth" drugs, and this familiarity would help to reduce the effects of harassment by the interrogator before and after the drug has been administered. From the viewpoint of the intelligence service, the trial exposure of a particular operative to drugs might provide a rough benchmark for assessing the kind and amount of information he would divulge in narcosis.

There may be concern over the possibility of drug addiction intentionally or accidentally induced by an adversary service. Most drugs will cause addiction with prolonged use, and the barbiturates are no exception. In recent studies at the U.S. Public Health Service Hospital for addicts in Lexington, Ky., subjects received large doses of barbiturates over a period of months. Upon removal of the drug, they experienced acute withdrawal symptoms and behaved in every respect like chronic alcoholics.

Because their action is extremely short, however, and because there is little likelihood that they would be administered regularly over a prolonged period, barbiturate "truth" drugs present slight risk of operational addiction. If the adversary service were intent on creating addiction in order to exploit withdrawal, it would have other, more rapid means of producing states as unpleasant as withdrawal symptoms.

The hallucinatory and psychotomimetic drugs such as mescaline, marihuana, LSD-25, and microtine are sometimes mistakenly associated with narcoanalytic interrogation. These drugs distort the perception and interpretation of the sensory input to the central nervous system and affect vision, audition, smell, the sensation of the size of body parts and their position in space, etc. Mescaline and LSD-25 have been used to create experimental "psychotic states," and in a minor way as aids in psychotherapy.

Since information obtained from a person in a psychotic drug state would be unrealistic, bizarre, and extremely difficult to assess, the self-administration of LSD-25, which is effective in minute dosages, might in special circumstances offer an operative temporary protection against interrogation. Conceivably, on the other hand, an adversary service could use such drugs to produce anxiety or terror in medically unsophisticated subjects unable to distinguish drug-induced psychosis from actual insanity. An enlightened operative could not be thus frightened, however, knowing that the effect of these hallucinogenic agents is transient in normal individuals.

Most broadly, there is evidence that drugs have least effect on well-adjusted individuals with good defenses and good emotional control, and that anyone who can withstand the stress of competent interrogation in the waking state can do so in narcosis. The essential resources for resistance thus appear to lie within the individual.

CONCLUSIONS

The salient points that emerge from this discussion are the following. No such magic brew as the popular notion of truth serum exists. The barbiturates, by disrupting defensive patterns, may sometimes be helpful in interrogation, but even under the best conditions they will elicit an output contaminated by deception, fantasy, garbled speech, etc. A major vulnerability they produce in the subject is a tendency to believe he has revealed more than he has. It is possible, however, for both normal individuals and psychopaths to resist drug interrogation; it seems likely that any individual who can withstand ordinary intensive interrogation can hold out in narcosis. The best aid to a defense against narco-interrogation is foreknowledge of the process and its limitations. There is an acute need for controlled experimental studies of drug reaction, not only to depressants but also to stimulants and to combinations of depressants, stimulants, and ataraxics.

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Senator WALLOP. If they are, I would assume that you would still try to find from either theirs or somebody else's information how to protect our people from that kind of activity.

Admiral TURNER. Yes.

Senator WALLOP. Thank you very much. Thank you, Mr. Chairman.

Senator INOUE. Senator Chafee?

Senator CHAFEE. Thank you, Mr. Chairman.

Admiral Turner, I appreciate that these tawdry activities were taking place long before your watch, and I think you have correctly labeled them as abhorrent, but not only were they abhorrent, it seems to me they were rather bungled, amateurish experiments that don't seem to have been handled in a very scientific way, at least from the scanty evidence we have.

It seems to me that there were the minimum of reports and the Agency didn't have the ability to call it quits. It went on for some 12 years, as you mentioned. What I would like to get to is, are you convinced now in your Agency that those scientific experiments, legitimate ones that you were conducting with polygraph and so forth, were being conducted in a scientific manner and that you are handling it in a correct manner to get the best information that you are seeking in the end?

Admiral TURNER. Yes, I am, and I also have a sense of confidence that we are limiting ourselves to the areas where we need to be involved as opposed to areas where we can rely on others.

Senator CHAFEE. I am convinced of that from your report. I just do hope that you have people who are trained in not only handling this type of experiment, but in preparing the proper reports and drawing the proper data from the reports. You are convinced that you have this type of people?

Admiral TURNER. Yes, sir.

Senator CHAFEE. The second point I am interested in was the final lines in your testimony here, which I believe are very important, and that is that the Agency is doing all it can in cooperation with other branches of the Government to go about tracking down the identity of those who were in some way adversely affected, and see what can be done to fulfill the government's responsibilities in that respect. I might add that I commend you in that, and I hope you will pursue it vigorously.

A hospital in my State was involved in these proceedings, and it is unclear exactly what did take place, so I have both a parochial interest in this and a national interest as well, and I do hope you will press on with it. It involves not only you, I appreciate, but also HEW and perhaps the Attorney General.

Admiral TURNER. Thank you, sir. We will.

Senator CHAFEE. Thank you. Thank you, Mr. Chairman.

Senator INOUE. Thank you very much.

Admiral Turner, MKULTRA subproject 3 was a project involving the surreptitious administration of LSD on unwitting persons, was it not?

Admiral TURNER. Yes, sir.

Senator INOUE. In February 1954, and this was in the very early stages of MKULTRA, the Director of Central Intelligence wrote to the technical services staff officials criticizing their judgment because they had participated in an experiment involving the administration of LSD on an unwitting basis to Dr. Frank Olson, who later committed suicide. Now, the individuals criticized were the same individuals who were responsible for this subproject 3, involving exactly the same practices. Even though these individuals were clearly aware of the dangers of surreptitious administration and had been criticized by the Director

of Central Intelligence, subproject 3 was not terminated immediately after Dr. Olson's death.

In fact, according to documents, it continued for a number of years. Can you provide this committee with any explanation of how such testing could have continued under these circumstances?

Admiral TURNER. No, sir, I really can't.

Senator INOUE. Are the individuals in the technical services who carried on subproject 3 still on the CIA payroll?

Admiral TURNER. I am sorry. Are you asking, are they today?

Senator INOUE. Yes.

Admiral TURNER. No, sir.

Senator INOUE. What would you do if you criticized officials of the technical services staff and they continued to carry on experimentation for a number of years?

Admiral TURNER. I would do two things, sir. One is, I would be sure at the beginning that I was explicit enough that they knew that I didn't want that to be continued anywhere else, and two, if I found it being continued, I would roll some heads.

Senator INOUE. Could you provide this committee with information as to whether the individuals involved had their heads rolled?

Admiral TURNER. I don't believe there is any evidence they did, but I will double check that.

[See p. 170 for material referred to.]

Senator INOUE. As you know, Senator Huddleston and his subcommittee are deeply involved in the drafting of charters and guidelines for the intelligence community. We will be meeting with the President tomorrow. Our concern is, I think, a basic one. Can anything like this occur again?

Admiral TURNER. I think it would be very, very unlikely, first, because we are all much more conscious of these issues than we were back in the fifties, second, because we have such thorough oversight procedures. I cannot imagine that this kind of activity could take place today without some member of the CIA itself bypassing me, if I were authorizing this, and writing to the Intelligence Oversight Board, and blowing the whistle on this kind of activity.

I am also doing my very best, sir, to encourage an openness with myself and a free communication in the Agency, so that I am the one who finds these things if they should happen. The fact is that we must keep you and your committee and now the new committee in the House informed of our sensitive activities. I think all of these add up to a degree of scrutiny such that this kind of extensive and flagrant activity could not happen today without it coming to the attention of the proper authorities to stop it.

Senator INOUE. A sad aspect of the MKULTRA project was that it naturally involved the people who unwittingly or wittingly got involved in experimentation. I would appreciate it if you would report back to this committee in 3 months on what the Agency has done to notify these individuals and these institutions, and furthermore, to notify us as to what steps have been taken to identify victims, and if identified, what you have done to assist them, monetarily or otherwise.

Admiral TURNER. All right, sir. I will be happy to.

Senator GOLDWATER. Will the Senator yield?

Senator INOUE. Yes, sir.

Senator GOLDWATER. I wonder if he could include in that report for our information only a complete listing of the individuals and the experiments done on them, and whether they were witting or unwitting, volunteer or nonvolunteer, and what has been the result in each case. I think that would be interesting.

Admiral TURNER. Fine. Yes, sir.

Senator INOUE. Senator Kennedy?

Senator KENNEDY. Thank you. It is your intention to notify the individuals who have been the subjects of the research, is that right, Admiral Turner? Do you intend to notify those individuals?

Admiral TURNER. Yes.

Senator KENNEDY. If you can identify them, you intend to notify them?

Admiral TURNER. Yes.

Senator KENNEDY. And you intend to notify the universities or research centers as well?

Admiral TURNER. Senator, I am torn on that. I understand your opening statement. I put myself in the position of the president of one of those universities, let's say. If he were witting—if his university had been witting of this activity with us, he has access to all that information today. If he were not witting, I wonder if the process of informing him might put his institution's reputation in more jeopardy than letting them go on the way they are today, not knowing. I really don't know the equities here.

Senator KENNEDY. Well, the problem is, all you have to do is pick up the newspapers and you see these universities mentioned. In many instances, I think you are putting the university people at an extraordinary disadvantage, where there is a complete change of administration, and they may for one reason or another not have information that they are under suspicion. There is innuendo; there is rumor. I cannot help but believe that it will just get smeared all over the newspapers in spite of all the security steps that have been taken.

It seems to me that those universities should be entitled to that information, so that the ones with other administrations can adapt procedures to protect those universities. The importance of preserving the independence of our research areas and the communities seems to me to be a very fundamental kind of question about the protection of the integrity of our universities and our research centers.

Admiral TURNER. You are saying that you feel that if we identify them privately to themselves, we can benefit them in an adequate way to cover the risk that this will lead to a more public disclosure? There are lots of the 80 who have not been identified publicly at this point.

Senator KENNEDY. I think the universities themselves should be notified. I think then the universities can take whatever steps in terms of their setting up the procedures to protect their own kinds of integrity in terms of the future. I would certainly hope that they would feel that they could make a public comment or a public statement on it. I think it is of general public interest, particularly for the people that are involved in those universities, to have some kind of awareness of whether they were used or were not used and how they were used.

I think they are entitled to it, and quite frankly, if there is a public official or an official of the university that you notify and he wants